RegenMed

RESPONSE TO CMS/ASTP/ONC REQUEST FOR INFORMATION HEALTH TECHNOLOGY ECOSYSTEM (RIN 0938-AV68)

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ABOUT REGENMED

RegenMed is a product agnostic company led by experienced physicians, medical scientists and businessmen. The Company partners with healthcare stakeholders in the collection, aggregation, and conversion of longitudinal real world clinical data into evidence-based standards of care.

Our Circles platform and associated processes were developed from the ground up over the past several years to accommodate the specific challenges identified in this Request for Information:

- Driving substantial cost reduction in the collection of clinically and statistically significant healthcare datasets.
- Supporting genuine outcomes-based value-based care correlated to specific clinical hypotheses, medical products, treatment protocols, and patient cohorts.
- Minimizing provider burden and cost relating to data collection in accordance with Good Clinical Practice, while delivering ethical non-biased incentives for doing so. (See this 3.5 minute video on Physicians-Owned Circles.)
- Secure 24/7 cloud-based access to longitudinal, clinically-relevant datasets from any device.
- Excellent patient user experience. Each patient can compare his long-term outcomes against his original baseline, as well the outcomes of similarly-situated patient cohorts. Longitudinal and clinically specific datasets relevant for and accessible to any given patient cohort supports SODH and health equity. A patient's longitudinal Case can accommodate any number of treatments for the same pathology by any number of practitioners wherever located.
- Circles datasets avoid the gaps, clinical irrelevance, high cost, complexity, disputed data ownership, opaque algorithms, data manipulation, AI hallucinations, lack interoperability, and other deficits associated with "big data" platforms.
- Circles enable physicians in smaller clinical settings, as well is in larger academic medical hospitals, to conduct high-quality medical research with evidence-based and accelerated clinical impact for all patient cohorts.
- Circles can address Complementary and Alternative Medicine (CAM) – estimated to be used by 40% of Americans – as well as more traditional protocols (which often have limited if any quality evidentiary support.)

GENERAL COMMENTS

High Quality Real-World Data

The most clinically relevant evidence of safety, efficacy, and value for any medical product or protocol is to be found in high quality real world datasets. Such primary datasets are in stark contrast to synthetic compendia of EMR, claims, prescription, revenue cycle management codes and

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other "big data" solutions.

High quality real-world datasets are defined by a single anatomical region, pathology, treatment protocol, and long terms outcome assessment. They are verifiable to primary sources – treating physicians and their patients -- and are inherently structured to deliver causal correlations among clinical hypothesis, treatments, and long-term outcomes for specific patient cohorts.

There are over one billion patient physician interactions each year in the U.S. Each of these represents longitudinal real-world data far more relevant for clinical decision making and VBC than reimbursement codes, drug prescriptions, or insurance claims. Modern technology enables the generation of genuine real world evidence for any particular health/wellness approach, rivaling in quality and clinical relevance that of much more expensive clinical trials.

High Quality RWE Represents Impactful Medical Research

The large majority of clinical trials today can be afforded only by well-capitalized pharmaceutical and medical device companies. Also, many peer-reviewed published studies are coming under increasing scrutiny for alleged bias. Even when such bias does not exist, the underlying raw data is rarely available. Medical literature (despite its accelerating volume) fails to address many if not most health/wellness issues in a clinically relevant and timely manner. In addition, NIH and other governmental institutions are substantially reducing research grants.

Meanwhile, physicians are -- as they have always been -- the most important source of clinical hypotheses which can lead to innovative products, diagnostic tools, and treatment protocols having a broad impact for all Americans. Those hypotheses arise in every day medical practices. Critically, the data needed to test those hypotheses arise in those same settings.

Impact of Recent Regulations and Rules.

As indicated in this RFI, the past years have seen a number of well-intentioned legislative and rule-making attempts to ensure the deployment of "effective digital capabilities needed to make informed health decisions, and increasing data availability for all stakeholders contributing to health outcomes". We respectfully submit that Circles can make a major difference in achieving those goals – with minimal cost and excellent patient and provider user experience.

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SPECIFIC RESPONSES

PATIENT NEEDS

PC-1

RFI: What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

Response: One which tracks long-term outcomes correlated to the specific treatment protocol or product which has been prescribed for the patient. One which allows her to compare those outcomes to her pre-procedure baseline, as well as against the aggregated outcomes of similarly situated patient cohorts. One which integrates, without burden or expense to her, the involvement of multiple caregivers along her full treatment journey. One through which she is comfortable her personal health information is not being sold to others, even on an anonymized basis.

RFI: What are the top things you would like to be able to do for you or your loved ones' health that can be enabled by digital health products?

Response: Provide independent, validatable evidence that the product for treatment protocol prescribed for is indeed, safe and efficacious overtime, ideally compared to alternative approaches. Recognize the potentially systemic nature of the patient's pathologies, while correlating treatment protocol and/or product data to those specific indications. Allows the patient, her caregiver and her healthcare provider to access clinically relevant longitudinal datasets relating to her diagnosed condition and potential treatment protocols.

RFI: If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.

Response: In addition to the foregoing, use modern AI learning models to query validated datasets to help the patient and his physician identify key questions and alternative treatment protocols, including adjunct and adjuvant therapies, related to the patient's specific condition.

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PC-2

RFI: Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

Response: In our experience, this is rare for the majority of patients, especially over any meaningful period of time. Despite well-intentioned laws and regulations, it is generally complex, difficult and often expensive for patients to aggregate their various medical data, especially in a clinically meaningful manner.

RFI: What apps should exist but do not yet? Why do you believe they do not exist yet?

Response: An app which integrates for a patient all clinically relevant information relating to a specific (i) diagnosis, (ii) treatment protocol and (iii) long-term standardized outcomes assessments. Each longitudinal dataset is a "Case", and the app would track all Cases for the patient regardless of provider(s), insurance plan(s), and primary and ancillary procedures. The app would allow the patient to compare his outcomes scores not only against his baseline, but against the averages of similar patient cohorts wherever located. See Benchmarc™.

It is not in the financial interests of EMR, RCM, or other healthcare data software companies to enable the easy transfer to physicians or to their patients of comprehensive and correlated healthcare information.

RFI: What set of workflows do you believe CMS is uniquely positioned to offer?

Response: Those which reflect the capabilities and user experience articulated above.

PC-5

RFI: What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

Response: Require providers to make patients aware of their availability. Established outcomes based registries making available clinically and statistically significant

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datasets based on primary validatable data, each such registry representing a single (i) anatomical region, (ii) pathology, (iii) treatment protocol, and (iv) standardized outcome assessments.

RFI: What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)? What changes would enable timely access to high quality CMS and provider generated data on patients?

Response: CMS should stand for quality and accessibility of healthcare data used by Medicare patients and their physicians for clinical decision-making and value-based care. That means data source transparency, verifiability, lack of bias, clinical and statistical significance for all patients and treatment protocols, low or no cost for access to such data, and excellent user experience in the context of such access.

PC-6

RFI: What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

Response: No cost or complexity for patients. Direct relevance to a patient's specific condition, treatment protocol, and outcomes. Easy establishment of access protocols, while maintaining full security. Since most products will be delivered through providers, human support from technology vendors for providers and patients. Easy access by caregivers, while maintaining security and identification of data sources.

PC-7

RFI: If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

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Response: By supporting the use of Circles, or another platform with similar capabilities, cost, and patient user experience.

PC-8

RFI: In your experience, what health data is readily available and valuable to patients or their caregivers or both?

Response: Reimbursement, disease, prescription, and other codes are largely irrelevant to patients, as is claims data. More potentially relevant are demographic information, blood pressure, lab tests, scans, diagnoses, and clinical intervention data. However, even this data is of limited value to patients since it is poorly presented, and the patient is unable to make meaningful correlations among her specific conditions, recommended treatment protocols, and long-term outcomes.

RHI: What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?

Response: Standardized outcomes data relating to the patient's specific diagnosis and treatment protocol. Comparison of such outcomes data against that of similar patient cohorts. Information on the clinical/scientific basis for the treatment protocol prescribed, including differential diagnoses, and alternative, adjunct and/or adjuvant therapies considered.

RFI: What are specific sources, other than claims and clinical data, that would be of highest value, and why?

Response: Aggregated, longitudinal and well-structured real world data based on similar pathologies, treatment protocols, and patient cohorts would be of the highest clinical value. It combines the rigor of traditional clinical trials with much larger statistical samples, and is constantly and automatically improved through ongoing data collection and analysis.

RFI: What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable

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more meaningful clinical research and generation of actionable evidence?

Response: Technically, interoperability should not be difficult. FHIR and other protocols have been long available for standardization of healthcare data. The lack of progress is due to insufficient incentives and, indeed, financial motivation to maintain ownership of data by various healthcare data platforms.

More actionable evidence for clinical decision-making and value-based care lies in the use of well-structured real world data in a manner closely approximating the rigor of clinical trials. This is the specific purpose of the underlying technology and associated processes of Circles.

PC-9

RFI: Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

Response: For genuine and actionable value-based care, one must collect long-term outcomes verifiably correlated to specific treatment protocols, pathologies and patient cohorts. Moreover, the resulting datasets must be clinically and statistically significant, transparent as to primary sources, and easily accessible to all relevant stakeholders.

"Big data" solutions have attempted for many years to develop algorithms purporting to generate approximations of such datasets by ingesting large amounts of EMR, RCM, eRX and claims data. This is and will remain a fundamentally flawed approach – even with AI – due to the data gaps, lack of clinical context, absence of outcomes, multiple (often unknown) forms of data manipulation, and inability to verify primary sources.

RFI: What difficulties are there in accessing or utilizing these data sources today?

Response: Such data sources are rare. On the one hand, the type of datasets which can lead to safer and more efficacious standards of care, to faster evidence-based lab to bedside medical innovations, to better, lower-cost and personalized medicine for more people is well known. It s the type of datasets generated by rigorous clinical trials. On the other hand, such trials have become so burdensome and expensive that only large for-profit companies can afford them.

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The solution is to take advantage of modern technology and ethical incentives to generate such datasets from the large amounts of real-world and clinically relevant data inherent in everyday medical interventions, coupled with the capture of long-term outcomes data specifically tied to those interventions.

RFI: What suggestions do you have to improve the Blue Button 2.0 API experience?

Response: From the physician's perspective, implement the type of UX found here. From the patient's perspective, here.

RFI: Is there non-CMS data that should be included in the API?

Response: Yes, the types clinically-specific datasets described in the foregoing Responses.

PC-10

RFI: How is the Trusted Exchange Framework and Common Agreement™ (TEFCA™) currently helping to advance patient access to health information in the real world?

Response: Cost, technical complexity, and limited motivation will likely continue to hinder the implementation of TEFCA by all except the larger provider groups. The more important issue is the weak clinical relevance and applicability to genuine value-based care inherent in the datasets which are "exchanged" through TEFCA or similar platforms.

PC-11

RFI: How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

Response: Very little due to the general lack of (i) clinical relevance, (ii) outcomes data, (iii) ability to validate the presented information, and (iv) applicability to true value-based care.

PC-12

RFI: What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

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Response: Transparent and statistically-significant outcomes data tied to specific clinical interventions, products and providers. The ability to compare such outcomes data across various treatments and product for the same pathology and patient cohort. This is possible today through Circles.

Information Blocking and Digital Identity

PC-14

RFI: Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 credentialing service providers (CSP). What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials? What could be the benefits to patients/caregivers if digital identity credentials were more widely used?

Response: A longitudinal dataset for a single patient, comprising validated data from the initial diagnosis and treatment protocol through long terms outcomes, represents the digital core of a healthcare "token". That token – and therefore all of that data – can be secured through public/private key technology used for crypto-currencies – a much higher level of security than is currently used for PHI. (This is particularly important as PHI increasingly includes entire genomic and other "omic" data.)

Moreover, that token can be (for example) exchanged for lower health insurance premiums, access to proprietary clinical datasets relevant to the patient's specific pathology, and other purposes. RegenMed is currently developing such secure token technology for patients and physicians using its Circles platform.

PROVIDERS

Digital Health Apps

PR-1

RFI: What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

Response: Explicitly support processes which (i) eliminate the burden associated with collecting, aggregating, and submitting clinically relevant data, (ii) enable providers wherever located to access indication- and protocol-specific real-world evidence datasets generated by their peers around the country, (iii) are free to low-cost to access, (iv) are regularly updated with additional relevant cases and as well as longer-term outcomes from existing cases.

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PR-2

RFI: What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

Response: The obstacles are cost, burden, poor user experience for both patients and clinicians, and limited if any relevance to the specific clinical situation facing the patient and her physician.

PR-3

RFI: How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:

Response: It is very important. Otherwise, it is impossible to achieve being full health equity or develop effective products and treatment protocols in the context of social determinants of health. The lack of access by clinicians in urban and rural areas to standards of care developed in academic medical centers is a major reason for poor health metrics among many patient groups.

PR-4

RFI: What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment?

Response: With modern technology, enabling patient access to administrative workflows and scheduling. The more pressing issue is enabling the generation of and accessibility to clinically relevant real-world datasets to drive better and lower-cost standards of care across all therapeutic areas.

Data Exchange

PR-5

RFI: Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

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Response: Our APIs currently support several FHIR modules, and we continually expand that support.

PR-6

RFI: Is TEFCA currently helping to advance provider access to health information?

Response: Not broadly, especially outside of large hospital systems. Moreover, data exchanges are only as useful for clinical decision-making and value-based care as the quality of the data made available. Generally, that data is of limited value due to lack of correlated outcomes, incomplete clinical context, statistical weakness, bias, inability to verify data, data "cleansing" and other manipulations.

PR-7

RFI: What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

Response: Healthcare data companies have the ability to achieve inter-operability, if there was a financial incentive to do so. However, the incentives are to the contrary. Those companies earn significant license fees from "their" data.

By supporting platforms such as Circles, CMS and its partners can help providers, patients and other healthcare stakeholders access real-world evidence datasets which are clinically and statistically significant, unbiased, longitudinal and continually refreshed with well-correlated and verifiable data.

PR-8

RFI: What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

Response: Support platforms and processes which minimize burden of, and provide ethical incentives for, high quality real-world data collection in the context of everyday medical interventions, and deliver meaningful clinical value to providers and their patients.

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Digital Identity

PR-9

RFI: How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

Response: Properly structured, a patient's own longitudinal healthcare data is the foundation its most distinct and secure identity credentials. See the response to PC 14 above. Clear, ID – me login.gov and other third-party credentialing approaches will not be applicable to all, or even perhaps the majority of the population. Conversely, by definition, each patient has its medical record.

Moreover, patients are increasingly aware of the value of their personal health data, whether or not it is anonymized. This awareness will only grow as PHI includes such personally complete descriptors as genomics, microbiology, and other systemic data. (See the recent and controversial 23 & Me Chapter 11 sale of genomic data.)

The bigger issue, therefore, is not only providing higher quality security for that data, but inducing patients to allow its use beyond their personal care. A single answer to both issues lies in the use of modern public key/driver key cryptographic tokenization based on the patient's unique longitudinal healthcare dataset. See our White Paper on this topic.

PAYERS

PA-1

RFI: What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?

Response: See our Response to PR-6.

PA-2

RFI: How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

Response: First, ensure that the platform delivers trustworthy and meaningful clinical value to patients and physicians in the context of specific pathology, treatment

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protocols, and patient cohorts. Second, provide a modern intuitive user interface. Third, ensure that physicians and patients are appropriately incentivized to contribute data sets to the platform which represent the clinical value.

PA-3.

RFI: How can CMS encourage payers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins?

Response: See our Response to PR-9.

PA-4.

RFI: What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

Response: The value of any directory is proportional to the utility it provides its clinician and patient users. That utility is defined by how well the user can access data, allowing him to identify primary care or specialists demonstrating above average long-term outcomes in the context of specific pathologies, treatment protocols, and patient cohorts.

Whether those clinically and statistically relevant data sets are presented through FHIR endpoints, or other trustworthy structures, is relatively unimportant compared with their quality and clinical significance.

PA-5

RFI: What are ways payers can help with simplifying clinical quality data responsibilities of providers?

Response: Support systems which (i) collect and aggregate well-structured longitudinal datasets for specific conditions and treatments, (ii) are free to low cost for physicians and patients, (iii) represent no interruption to normal clinical flow for physicians, and (iv) ethically motivate physicians and patients to contribute to those datasets.

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TECHNOLOGY VENDORS, DATA PROVIDERS, AND NETWORKS

Ecosystem

TD-1

RFI: What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Response: Extend to other pathologies initiatives such as CMS Inpatient THA/TKA Pro-PM. Implement value-based care policies which extend beyond re-admission rates and similarly limited measures to identify genuine correlations between specific treatments/products and well-correlated long-term outcomes. Implement SDOH initiatives to incentivize the availability to rural, community and other providers and patients of evidence-based standards of care achievable through the efficient and economic analysis of real-world data.

Coordinating with the FDA, distinguish between real-world evidence based on expensive and clinically-weak big data solutions on the one hand and, on the other hand, real-world datasets which are clinically and statistically significant, include well-correlated long-term outcomes, independent, and validatable to primary sources.

Digital Identity

TD-3

RFI: Regarding digital identity implementation: What are the challenges and benefits? How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange? What impact would mandatory use of the OpenID Connect identity protocol have?

Response: See our Response to PR-9.

Technical Standards and Certification

TD-4

RFI: How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

Response: Explicitly identify such APIs for access by all healthcare stakeholders. Provide comparative advantages and disadvantages for providers and patients. Where

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appropriate provide access to resultant datasets/registries for specific pathologies, treatment protocols, products, patient cohorts with indications of potential bias, gaps or other limitations on reliance.

TD-5

RFI: Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

Response: Use of learning models and other forms of AI is an inevitable direction for healthcare. The key is to fully indicate data sources and the nature of data manipulation inherent in any dataset or derived correlation.

TD-9

RFI: What policy changes could CMS make so providers are motivated to respond to API-based data requests with best possible coverage and quality of data? How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?

Response: In the context of any dataset used for clinical decision-making or value-based care, "best possible coverage" should be defined to encompass (at least in part) clinical relevance across common pathologies, treatment protocols, products, patient cohorts.

"Data quality" should be defined to include parameters found in clinical trial studies such as as long-term outcomes causally correlated to specific clinical/scientific endpoints, statistical significance, validatable primary data sources, proper informed patient consents for data use/monetization, and indications of adherence to other Good Clinical Practice criteria.

TD-10

RFI: For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification (42 U.S.C. 300jj-11(c)(5)(D)(iv)) that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

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Response: Allow/require a CMS-sponsored "sandbox" for developers, providers, and payers to, without cost, test access to sample longitudinal health data.

Data Exchange

TD-12

RFI: Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

Response: CMS should endorse non-CMS data sources and networks which meet the best possible coverage and data quality criteria identified in the Response to TD-9, above.

TD-13

RFI: What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

Response: See Response to PR-9, above.

TD-17

RFI: Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand that results in usage and outcomes?

Response: CMS and ASTP/ONC should (i) identify the of healthcare data sharing networks delivering datasets meeting the "best possible coverage" and "data quality" criteria identified in the Response to TD-9 above, and (ii) offer preferred reimbursement programs to providers submitting such datasets.

Compliance

TD-19

RFI: Regarding price transparency implementation: What are current shortcomings in content, format, delivery, and timeliness? Which workflows would benefit most from functional price transparency? What improvements would be most valuable for patients, providers, or payers, including CMS? What would further motivate solution development?

Response: Information blocking is common. Common forms of such blocking are high fees, unnecessarily complex technical requirements, delay in granting access. Moreover, these blocking costs and technical complications are a direct

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consequence of the ultimately unsuccessful attempt to drive clinically useful information from diverse sources of "big data".

The focus should be on identifying, in the context of specific pathologies and treatment protocols, those relatively few medical/scientific data points which carry real significance for clinical decision-making and value-based care. (Generally, these are not reimbursement or prescription codes.)

With these key data categories identified, it is then a matter of efficiently collecting the corresponding data – including correlated long-term outcomes -- in real world settings according to GCP standards and with minimal clinical burden. By thus simplifying the generation of datasets to their core beneficial components, it will be far easier to make accessible and exchange such datasets.

VALUE-BASED CARE ORGANIZATIONS

Digital Health Adoption

VB-1

RFI: What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

Response: As a first stage, APMs and MSSP participants should be subjected to reduced reimbursement for failure to collect long-term outcomes correlated to specific treatment protocols and patient cohorts, analogous to the CMS Inpatient THA/TKA Pro-PM. As a second stage, they should be subjected to reduced reimbursement for outcomes below average or other CMS-specified benchmarks.

VB-2

RFI: How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

Response: These should not be integrated into APM requirements as much as integrated into the definitions of "best possible coverage" and "data quality" (RFI TD-9),

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with a particular focus on ensuring datasets representing verifiable primary data sources to minimize the risk of error rates and other forms of "hallucinations".

VB-3

RFI: What are essential health IT capabilities for value-based care arrangements?

Response: Long-term outcomes capture correlated to specific pathologies, treatment protocols, products and standardized assessments. Non-biased, clinically relevant and statistically significant datasets fully verifiable to primary data sources. Excellent physician and patient experience to ensure maximum compliance. See Circles.

Technical Standards

VB-13

RFI: What improvements to existing criteria and standards would better support value-based care capabilities while reducing provider burden?

Response: Criteria for value-based care in any setting should focus on collection of long-term outcomes well correlated to specific pathologies, products, treatment protocols, and patient cohorts. Implementation of such criteria by patients and providers requires excellent user experience for both, including minimal clinical interruption for providers. Finally, providers will generally require ethical consideration for the development of such datasets, such as is offered in Physicians-Owned Circles.

VB-14

RFI: How could implementing digital identity credentials improve value-based care delivery and outcomes?

Response: Through offering patients adequate incentive to allow the use of their clinically relevant personal health datasets, as well as a high level of confidence in the security of those personal datasets. See Response to TD-9 above.

VB-15

RFI: How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources? What key data elements would be necessary in a nationwide FHIR endpoints directory to maximize its effectiveness?

Response: The key to such directories is the quality, clinical specificity, and verifiability of

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the underlying datasets. Quality depends on each dataset being relevant to a specific pathology, clinical intervention, and patient cohort, as well as including well-correlated long-term outcomes report against standardized assessments.

Verifiability requires datasets transparently derived from real world clinical data from primary sources – physicians, patients, laboratories. Those accessing and querying such datasets should be able easily to satisfy themselves as to data sources and lack of data manipulation. See our White Papers, Advantages Of Circles Over "Big Data" RWE, and Recursive AI Learning Model For Primary and Proprietary Healthcare Datasets.

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